

Question:

A newborn male is evaluated in the hospital nursery 24 hours after birth for cyanosis. The patient was born at 38 weeks gestation to a 36-year-old gravida 3 via cesarean section for fetal distress. The patient’s mother received inconsistent prenatal care, and the delivery was uncomplicated. The patient’s Apgar evaluation was notable for acrocyanosis at both 1 and 5 minutes of life. The patient’s mother denies any family history of congenital heart disease. The patient’s father has a past medical history of hypertension, and one of the patient’s older siblings was recently diagnosed with autism spectrum disorder. The patient’s birth weight was 3180 g (7 lb 0 oz). In the hospital nursery, his temperature is 99.3°F (37.4°C), blood pressure is 66/37 mmHg, pulse is 179/min, and respirations are 42/min. On physical exam, the patient is in moderate distress. He has low-set ears, orbital hypertelorism, and a cleft palate. The patient is centrally cyanotic. A chest CT shows thymic hypoplasia. Echocardiography demonstrates a single vessel emanating from both the right and left ventricle.

This patient should be urgently evaluated for which of the following acute complications?

Answer Choices:

- A. Cerebral edema
- B. Hypoglycemia
- C. Neuromuscular irritability
- D. Shortening of the QT interval

Reasoning + Answer:

<think> reasoning process here </think>< answer> C. Neuromuscular irritability </answer>

